

RE: MM LOT1 FU discussion

JM

Julia Moore

To

Onkar Kshirsagar

Retention Policy

GSK-Default - 13Months (1 year, 3 months)

Proprietary

Expires: 08-07-2027

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Wed 08-04-2026 02:13

From: Julia Moore

Sent: Monday, April 6, 2026 4:22 PM

To: Onkar Kshirsagar <onkar.s.kshirsagar@gsk.com>

Subject: MM LOT1 FU discussion

Hi Onkar,

Following up per our discussion last week, I have commented on your new spec, tab "10. LOT1_BASED_END OK" and have a left a number of comments we can go over this week. Regarding exploration of LOT1 profiles/validation, it would be helpful to look at the following:

1. Among the patients who receive a combination regimen for their LOT1 that contains a maintenance approved medication, is there a way to see what is happening with these individual agents outside the induction window? Are any of the maintenance-approved meds alone (dual meds alone together) for an obvious/standard period of time? I'm really interested in people who start with a combo regimen and then drop down to just a maintenance-approved med during LOT1, and how long they're on that maintenance med alone. Maybe if we can produce a Sankey of the MAPs that make up these regimens? Let's discuss. Maintenance-approved medication list:

a. Icenatidomide

b. Bortezomib

c. Daratumumab

d. Ixazomib

e. Thalidomide

f. Bortezomib/icenatidomide

g. Carfilzomib/icenatidomide

h. Daratumumab/icenatidomide

2. As discussed, can we see the patients who are receiving Cyclophosphamide monotherapy as their LOT1 regimen? Do these patients receive any SCTs soon after induction? Are we correctly identifying their diagnosis date (does changing the diagnosis date change this regimen)?

3. Make sure our starting cohort is among the ~21,000pts (not the ~8,000 where all criteria have been applied)

4. I see in the Dashboard under "LOT1 Base End Reasons" there is no one whose LOT1 is ending due to AUTO SCT. Is this up to date? If this is true, this makes our data quite clean/simple according to our EE. To discuss more on our next call.

5. Confirming no one had a CAR-T event interrupt their LOT1? Because we are rolling up CAR-T with any new agent introduced within 45 days (was 30d) of a patient initiating CAR-T, I think we may not yet have the full picture yet as to if someone discontinued LOT1 due to CAR-T.

Thank you,

Julia Moore, MS

Associate Director, Oncology

Real-World Biostatistics (RWB)

E: [julia.ymore@gsk.com](mailto:julia.ymoore@gsk.com)

M: +1 925.461.8944

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 Julia Moore
To: Onkar Kshirsagar
Retention Policy GSK-Default-13Months (1 year, 3 months)
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Hi Onkar,

have added a file called [Mix_scenarios.xlsx](#) to the study folder to provide some example case scenarios of what we'd like to explore for maintenance regimens. Let me know what you think as far as a potential visualizer to look at these patients.

Next time we meet, if possible, I'd like to have a draft of LOT1 ready. Next steps then mean:

1. Incorporating SCT events into LOT1 end reasons
2. Ensuring new agents that interrupt LOT1 that have a CART event within 45 days are properly flagged as CART consolidation therapy
3. Ensure the patient cohort is amongst the ~21,000
4. Other investigational to do's are detailed in my email below (though bringing LOT1 to a finish is the main priority for now).
5. Regarding the 60day extension flag, that variable was carried over from the prior spec and I will remove that part from the protocol. That was providing a reason to extend the LOT if there was SCT near the defined LOT1 discontinuation date (I have hid those rows in the spec)

Thanks,
Julia

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- a. Lenalidomide
- b. Bortezomib
- c. Daratumumab
- d. Isaxozomib
- e. Thalidomide
- f. Bortezomib/lenalidomide
- g. Carfilzomib/lenalidomide
- h. Daratumumab/lenalidomide

I